

August 12, 2026

Medicare (Palmetto GBA, JM MAC)
Provider Appeals Department

RE: Formal Appeal of Claim Denial — Loria Jenkins (MRN MUSC8620970), Claim MUSC-20250627-E7DE-2

Patient	Loria Jenkins (DOB 1937-07-15)
MRN	MUSC8620970
Member ID / Group	MED286101916 / GRP-47725
Claim number	MUSC-20250627-E7DE-2
Date of service	2025-06-27
Procedure billed	CPT 93306 — Transthoracic echocardiography, complete
Primary diagnosis	I25.10 — Atherosclerotic heart disease of native coronary artery
Denied amount	\$1,669.47
Denial code	CARC 50 / RARC N115 — These are non-covered services because this is not deemed a 'medical necessity' by the payer.
Appeal deadline	2025-11-11

Dear Appeals Review Committee:

SUMMARY OF APPEAL

MUSC Health is submitting this first-level redetermination request to Palmetto GBA (Jurisdiction M MAC) on behalf of patient Loria Jenkins (MRN: MUSC8620970, Member ID: MED286101916) regarding Claim ID MUSC-20250627-E7DE-2, date of service June 27, 2025. The claim was submitted on June 29, 2025, for CPT 93306 (transthoracic echocardiography, complete), billed at \$1,669.47, and denied in full on July 14, 2025. The payer cited CARC 50 ("These are non-covered services because this is not deemed a 'medical necessity' by the payer") and RARC N115 ("This decision was based on a Local Coverage Determination (LCD)"), with the accompanying remark that "Documentation submitted does not establish that the service was reasonable and necessary for the diagnosis reported." MUSC Health respectfully disagrees with this determination and submits the following clinical and coding evidence in support of reversal.

CLINICAL BACKGROUND

Ms. Jenkins is an 89-year-old female with a longstanding, active diagnosis of atherosclerotic heart disease of native coronary artery (ICD-10 I25.10), which has been documented in her medical record since at least July 1992. She carries concurrent active diagnoses of hyperlipidemia (ICD-10 E78.5) and prediabetes (ICD-10 E11.9), both of which are recognized comorbidities that compound cardiovascular risk and disease progression. Her cardiac history is substantive: she has been maintained on furosemide (both oral and injectable formulations), metoprolol succinate extended-release, and Lasix — a medication regimen consistent with ongoing management of cardiac dysfunction. A prior transthoracic echocardiogram performed in August 1992 documented a left ventricular ejection fraction of approximately 45%, indicating borderline reduced systolic function at that time. A subsequent echocardiogram was performed in February 1993. The June 27, 2025 study represents clinically appropriate longitudinal reassessment of a patient whose cardiac status, age, and active pharmacologic management warrant periodic echocardiographic surveillance.

BASIS FOR APPEAL

The denial asserts that the submitted documentation does not establish medical necessity for CPT 93306 under the applicable LCD. MUSC Health contends that this determination is not supported by the clinical facts of record. Ms. Jenkins presents with an active, chronic diagnosis of atherosclerotic heart disease (I25.10) — the primary diagnosis on the claim — in the context of a complex cardiovascular profile that includes hyperlipidemia, prediabetes, and a prior documented ejection fraction in the borderline range. She is actively prescribed multiple cardiac medications, including a loop diuretic and a beta-blocker, which are standard agents used in the management of structural heart disease and heart failure. Transthoracic echocardiography is a well-established, guideline-supported tool for evaluating cardiac structure and function in patients with known coronary artery disease, particularly when ongoing pharmacologic therapy necessitates periodic reassessment of ventricular performance. The rendering provider, Amara J. Boone, NP (NPI 1203948576), performed this service at MUSC Health East Cooper Medical Center, an on-campus outpatient hospital facility (Place of Service 22), within the scope of managing Ms. Jenkins' documented cardiac condition. The clinical indication is directly supported by the primary diagnosis, the patient's active medication regimen, and her established cardiac history. MUSC Health is prepared to submit the complete encounter documentation, including the ordering provider's clinical notes and the echocardiography report, in support of this appeal.

REQUESTED ACTION

MUSC Health respectfully requests that Palmetto GBA conduct a full redetermination of Claim ID MUSC-20250627-E7DE-2 and reverse the denial of CPT 93306 for the date of service June 27, 2025. We request that the claim be reprocessed and payment issued at the applicable Medicare allowed amount. Should additional clinical documentation be required to complete this review, please contact the MUSC Health Revenue Cycle Appeals team at your earliest convenience so that records may be provided promptly and within the applicable timeframe.

Respectfully submitted,

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Enclosures

- Remittance advice / explanation of payment
- Itemized claim detail (UB-04 / CMS-1500)
- Relevant clinical documentation from the medical record